



HEALTH INFORMATION MANAGEMENT — RELEASE OF INFORMATION

CERTIFICATION OF MEDICAL RECORDS

I, the undersigned, am the duly authorized custodian of records for Back & Spine Restoration Center and certify that the attached records, consisting of 4 pages, are true and complete copies of the medical records of:

PATIENT: Dax Vance

DATE OF BIRTH: 04/12/2001

MRN: 940-2281-A

DATES OF SERVICE: 2022-09-01 to 2022-10-31

These records were made and kept in the ordinary course of business at or near the time of the acts, conditions, or events described, and were produced pursuant to the authorization and request received from **Vance & Partners Injury Law**.

D. Okonkwo

Custodian of Records, Health Information Management

Date: 09/10/2023

**NEW PATIENT INTAKE QUESTIONNAIRE (COMPLETED BY PATIENT)**

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/10/2022

Name: Dax Vance DOB: 04/12/2001What brings you in today? Severe neck pain and stiffness after a heavy weightlifting gym accident last week.Pain right now (0-10): 6 Worst: 8 Best: 3Current medications: Advil as neededWhat treatments have you already tried? Ice packs at homeDo you smoke? NoAlcohol use: Occasional, 3-6 drinks per weekMarijuana / other substances: NoCurrent work status: ActiveHow is your sleep? Poor due to neck achePatient signature: Dax Vance Date: 09/10/2022

**SPINE & PM&R EVALUATION NOTE**

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/12/2022

SUBJECTIVE

Patient presents with persistent mid-cervical pain radiating into the bilateral trapezius. Pain began 6 days ago during a heavy squat set in the gym. Patient describes pain as a constant deep ache, rated 6/10. Denies upper extremity paresthesias or weakness.

OBJECTIVE

Cervical spine exam reveals tenderness to palpation from C4 through C6. Significant spasm of bilateral paraspinal muscles. Range of motion is restricted: flexion 30 degrees (normal 45), extension 15 degrees (normal 45). Bilateral upper extremity motor and sensory exam is intact.

ASSESSMENT

Acute cervical strain with possible underlying cervical disc herniation or bulge. Rule out disc pathology.

PLAN

Order cervical spine X-ray/MRI. Initiate conservative treatment with physical therapy, stretching, and nonsteroidal anti-inflammatory drugs. Patient to follow up in 2 weeks.

Dr. Alistair Finch

Dr. Alistair Finch, MD — Physical Medicine & Rehabilitation

NPI 1022931182 · Signed 09/12/2022

**DIAGNOSTIC IMAGING REPORT**

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/15/2022
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EXAMINATION	X-ray Cervical Spine
ACCESSION	PRIOR-RAD-2281 · Exam date 09/15/2022
INTERPRETING MD	Dr. Alistair Finch, MD

TECHNIQUE

Three-view cervical spine series.

FINDINGS

Cervical vertebral body heights are well-maintained. Mild degenerative narrowing of the disc space is observed at C5-C6. There is a 4mm posterior focal disc bulge at the C5-C6 level causing mild indentation of the anterior aspects of the thecal sac. No spinal canal stenosis or neuroforaminal narrowing seen.

IMPRESSION

1. Mild degenerative disc disease at C5-C6 associated with a 4mm posterior disc bulge. 2. No fracture or subluxation.



DAX VANCE · 940-2281-A · 09/15/2022 · PRIOR-RAD-2281
Prior C5-C6 disc bulge measuring 4mm

Dr. Alistair Finch

Dr. Alistair Finch, MD — Physical Medicine & Rehabilitation

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Electronically signed. Images archived on PACS under accession PRIOR-RAD-2281.